
Vaccine Diplomacy and Politics of Influence: India-China Competition and Bangladesh's Negotiated Autonomy in the COVID-19 Era

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Abstract

The COVID-19 pandemic turned vaccine distribution as the new frontier in the geopolitical competition, with the great powers wielding pharmaceutical assistance as a diplomatic instrument of soft power and political influence. This paper traces how India and China utilised vaccine diplomacy to pursue competing strategic interests in South Asia, specifically in Bangladesh. In the middle of the India-China rivalry, Bangladesh negotiated its sovereign interests through a well-calibrated multi-directional diplomatic strategy. The study is based on qualitative research methods, considering primary sources such as government statements, official briefs, and political leaders' speeches, as well as secondary sources such as existing literature, ORF reports, vaccination data, and media sources. The paper argues that India and China both differed significantly in scale, intent, and regional reception of vaccines in the world. India's Vaccine Maitri initiative projected New Delhi as the first responder and humanitarian partner in South Asia. However, China pursued a global, commercially driven strategy aimed at rehabilitating its international reputation following the Wuhan outbreak.

Bangladesh, being a passive recipient of this rivalry, faced increasing competitive pressure but demonstrated a considerable negotiated autonomy. By diversifying its vaccine sources from India, China, Russia and the COVAX multilateral facility, it leveraged bilateral competition and pursued domestic manufacturing deals in order to reduce long-term dependence. The findings indicate that developing states can strategically anchor great-power competition to advance their health security interests. Additionally, diplomatic gains from vaccine diplomacy remain dependent on sustained bilateral engagement rather than one-time provision.

Key Words: Geo-politics, Vaccine Diplomacy, Soft Power, COVID, Pandemic, India, China, Bangladesh, Vaccine Maitri, Vasudhaiv Kutumbakam

Introduction

The global political landscape is increasingly defined by the strategic pursuit of influence among superpowers and regional actors, with medical assistance emerging as a primary tool of soft power directed toward developing nations. During the COVID-19 pandemic, "vaccine diplomacy" became a central geopolitical instrument for major nations such as the United States, the United Kingdom, Russia, India, China, and international organizations. Southeast Asia, in particular, served as a critical theatre for these competing interests. India's tradition of *Vasudhaiv Kutumbakam* has been reflected in India's vaccine aid initiative, which won accolades for showing maturity at the times of crisis. Most nations lacked the technical knowledge and funds to make a vaccine, while some countries were not even in a position to buy the doses at market prices. This served as an opening for India to bridge the gap created in international politics. India actively exercised its "Neighbourhood First" policy and provided vaccines to regional partners directly challenging Chinese efforts to secure regional hegemony.

Amidst the helplessness of the United Nations and the World Health Organisation in dealing with power politics of major states like the USA and China, both India and South Africa put up a collaboration at the WTO and removed patent rights on raw materials for making vaccines. Further, the vaccines sent by China to Brazil entangled in political controversies view reportedly being tested on the Brazilian population with only 50 percent efficiency (Bhojraj, 2023). This led Brazil to request India and supply alternate Made-in-India vaccines. Thus, India's actions whilst supplying highly globally accepted vaccines portrayed India as 'solution creator'.

China was highlighted as a 'problem creator' in this pandemic view its engagement in vaccine diplomacy as only an extension of its established strategies of economic and geopolitical leverage, often characterized by the creation of external debt dependencies. In the context of the pandemic, these efforts were also interpreted as a strategic attempt to rehabilitate its international reputation following the initial viral outbreak in Wuhan, China.

Consequently, this research paper examines the "negotiated autonomy" of Bangladesh, analysing how the nation strategically navigated the competing diplomatic pressures of India and China. Often referred to as India's "all weather ally" in the neighbourhood, Bangladesh faced diplomatic challenges in balancing the strategic underpinnings during the vaccine assistance received from India and China (Bose, 2021). This study focuses on Bangladesh's ability to maintain its sovereign interests while managing the procurement of vaccines through both international donations and direct commercial purchases.

Problem Statement

How has Bangladesh negotiated between India and China through its vaccine diplomacy during COVID?

Literature Review

The theoretical foundations of vaccine diplomacy are rooted in the broader evolution of global health diplomacy as a field of study and practice. Kickbuski, Silberschmidt, and Buss (2007) define global health diplomacy as something that aims to capture the multi-level and multi-actor negotiation processes of "soft issues" of environment and health having "hard" ramifications, that shape and manage the global policy environment for health. They draw attention to the diversification of actors on various levels: the private sector, non-governmental organisations, scientists, activists, and the media, since all these actors are part and parcel of the negotiating process, and significance of the interaction between the diplomats and these actors. They further identify two types of imbalances that must be addressed: imbalances that can emerge between foreign policy and public health experts, and imbalances that exist in the negotiating power and capacity between developed and developing countries. Building on this foundation, Hotez (2014) introduces the foundational concept of vaccine diplomacy, suggesting that vaccines essentially function as "tools of soft power" capable of "reducing conflict" through scientific cooperation, and that vaccines historically allowed for "cooperation even between adversarial states." However, while Hotez views this through a humanitarian lens, contemporary developments in South Asia suggest that these tools are now being repurposed for competitive political influence. Van den Brink extends this theoretical grounding through the lens of Balance of Power theory, arguing that states use "soft power" to pull smaller nations into their coalition and away from a rival pole, and that the 2021 scarcity transformed vaccines into an "essential strategic resource," leading to a school of thought that recognises vaccine distribution as a purely political process aimed at maximising diplomatic gains. Sarker and Ghosh similarly argue that vaccine diplomacy in Bangladesh evolved from a "humanitarian necessity" into a "strategic survival mechanism," highlighting that for a middle power like Bangladesh, diplomacy was not just about health outcomes but about maintaining sovereign decision-making space amidst external pressures. Md. Emdadul Haque (2022) defines the environment in which this negotiated autonomy exists, describing a world of "scarcity of production, monopoly, [and] aggressive nationalism," and explicitly identifies "adhering to multilateralism in lieu of bilateralism" as the mechanism that allowed

Bangladesh to access diverse vaccine brands, thereby, supporting the argument that choosing the right type of diplomatic channel is itself a form of agency. Surianta and Dressel (2025) reinforce this by demonstrating that vaccine diplomacy is not a one-sided imposition by great powers, but rather a "negotiation process" where recipient countries actively influence outcomes, challenging the traditional view of small states as passive victims of rivalry and suggesting instead that the pandemic created opportunities for states to exert agency that existing literature often underestimates. Bawa Singh et al. describe global health diplomacy as an interdisciplinary concept linking health with international relations to address global health security while improving state relations, providing further theoretical grounding for understanding vaccine diplomacy as a permanent instrument of statecraft rather than a crisis-driven anomaly.

Within this theoretical framework, India's Vaccine Maitri initiative stands as the most prominent case of vaccine diplomacy in South Asia. Singh, Singh, Singh, and Chattu argue that India's vaccine diplomacy is rooted in the age-old philosophy of Vasudhaiva Kutumbakam ("the world is one family"), which has been a pivotal element of Indian foreign policy used to build a "peace-loving" international identity, and they explicitly link the Vaccine Maitri initiative to India's Neighbourhood First Policy, framing the delivery of the Covishield vaccine to SAARC countries as a "humanitarian view" that serves India's national interest. They highlight that while India's relations with neighbours had been "enervated" by internal and external dynamics, its pharmaceutical leadership acted as a tool to resurrect and strengthen these regional ties. Bose (2021) frames India's actions within its self-perceived role as the "net security provider of the region," noting that India's Neighbourhood First Policy drove its alacrity in responding to Bangladesh, and that vaccines were used to "improve the country's image" in Bangladesh after public opinion suffered due to the revocation of Article 370 and the CAA/NRC policies. She further notes that Bangladesh turned to India first as its "all-weather ally" due to shared civilizational links. Chakravarty argues that India's strategy is designed to remind neighbours that India remains the "first port of call in a disaster situation" and the primary "first responder" in the region, anchoring its foreign policy in "development cooperation" and harkening back to its formative years as a leader for newly independent Asian and African nations. As-Sazid captures the immediate diplomatic impact, noting that amidst issues like the Indian Citizenship Amendment Act 2019, the Teesta river's water sharing was a bottle-neck, and the unavailability of expected support for Rohingya repatriation (Sazid, 2022), India's initial support to Bangladesh with the provision of AstraZeneca vaccines was a significant diplomatic leap. Sarker and Ghosh document that Bangladesh initially relied almost exclusively on India's Vaccine Maitri through the Serum Institute of India, and that India's early

provision was a masterstroke of soft power that temporarily solidified India's status as a "first responder" and sidelined other competitors in the Bangladeshi market. Van den Brink finds, however, that while regional tensions exist, economic relations often provide a stronger statistical explanation for the variation in how many vaccines India provided to specific states than power tensions alone, offering an important qualification to purely geopolitical readings of India's motives.

The limits of India's soft power projection became starkly apparent when its domestic crisis created an opening for rival powers, transforming the regional vaccine landscape into what scholars broadly characterise as a geopolitical tug-of-war. Bose identifies that when India halted exports, it created a gap that China and Russia treated as an "opportunity to exert their own soft power," while As-Sazid describes the period more vividly: when India was hit by the Delta variant in March 2021, it created a "vacuum" that China and Russia immediately filled to "snap up the steering" of global diplomacy, characterising the Sino-Indian contest as a "scramble" and a "new diplomatic battleground" for soft power and global image-building. He captures the broader context memorably: "The Coronavirus Vaccine, one of the world's most coveted commodities, has become the new currency for international diplomacy. While the western world has resorted to staunch Vaccine Nationalism to preserve enough vaccines for their population, in the east, the race is for giving them away." Singh et al. note that India's quick launch of Vaccine Maitri reportedly took Beijing by surprise, prompting a "smear campaign" in Chinese media to discredit India's humanitarian narrative, and suggest that the competition is not just bilateral — the pandemic forced a "tectonic change" in the strategic landscape where major powers are now engaged in a "non-ideological worldview" of influence. Bose further highlights that Chinese diplomacy was not purely "soft": the Chinese ambassador warned Bangladesh that ties would be "damaged" if it joined the Quad alliance, and China capitalised on vaccine urgency to secure its "foothold in the Indian Ocean Region" and leverage the Belt and Road Initiative. Van den Brink identifies China and India as the two "poles" in Southern Asia, where each attempt to ensure the opposing coalition does not gain more states, while Chakravarty argues that vaccine diplomacy is explicitly designed to "create more space for India" as China pushes to expand its influence across South Asia. Bajpae and Jie (2025) situate this rivalry within a deeper structural conflict, arguing that it is not just about borders but is a deeper conflict between two "self-perceived civilization-states" seeking status and leadership in a multipolar global order, with China remaining "unwilling to recognize India as a peer and an equal," which constantly fuels New Delhi's frustration and the regional security dilemma. Antara Ghosal Singh (2025) adds that while military standoffs may temporarily end, the "political stalemate" between the two nations remains fundamentally unresolved, with China

insisting on continuing other aspects of the relationship despite border issues while India remains "steadfast in its position" that peace at the border is a prerequisite for any fruitful relationship in other sectors. Ian Tsung-yen Chen (2023) observes that the rivalry between Beijing and New Delhi is "most conspicuous" in neighbouring states and is expected to "potentially escalate in the foreseeable future," while also warning that any diplomatic gains achieved through vaccine aid may be "transitory in nature" and subject to reversal depending on domestic political transitions in the recipient state.

It is within this contested geopolitical environment that Bangladesh's response acquires its full significance, with the literature converging on a portrait of a state that exercised meaningful agency rather than passively absorbing the pressures of great-power competition. As-Sazid describes Bangladesh as "walking on a diplomatic tightrope" to maintain a balance between regional powers and extra-regional actors, dealing with "sheer pragmatism" to reap maximum benefits for its huge population and post-pandemic economic recovery. Bose captures the structural bind: after India's exports stopped, Bangladesh was "compelled to find alternative sources," moving from bilateral dependency to a broader search, and had to walk the "diplomatic tightrope" of balancing its strategic reliance on India with the practical necessity of accepting vaccines from China and Russia. Sarker and Ghosh document this in granular detail, identifying five elements of Bangladesh's autonomy: Strategic Diversification, moving from a single source (India) to a portfolio including China, Russia, and COVAX; Pragmatic Balancing, maintaining warm ties with India while simultaneously signing major deals with China; Active Multilateralism, leveraging the COVAX facility to reduce bilateral "political costs"; Domestic Manufacturing Ambition, negotiating for technology transfer such as the Sinopharm co-production deal to ensure long-term sovereignty; and Agency in Crisis, using the rivalry to negotiate better prices and faster delivery schedules, effectively turning great power competition into a "buyer's market" for Bangladesh. They note that Bangladesh ranked fifth globally in the Nikkei COVID-19 Recovery Index by May 2022, and that its proactive diplomacy and national preparedness plan allowed it to outperform many other nations in South Asia despite high population density and lower-middle-income status.

Haque highlights that Bangladesh was able to "harvest geopolitical dividends" from donor countries, in the context of the India-China rivalry, the "dividend" was the extra attention and aid Bangladesh received as a strategic state that everyone wanted to influence, and that Bangladesh received significant "cost-free gifts" from the United States alongside supplies from India and China. Singh et al. provide important context for Bangladesh's caution, noting that India's previous use of bilateralism in the neighbourhood was sometimes viewed by neighbours as "coercive diplomacy," which created early

apprehensions about over-reliance. Bajpae and Jie note that most South Asian countries have attempted to maintain a "balanced relationship with both China and India" to satisfy their own development needs, and that narratives about Chinese "debt traps" have had less seriousness in the region, including Bangladesh, because these states prioritise fulfilling their developmental requirements through any available partner. Surianta and Dressel observe a critical evolution in the diplomatic landscape from "product provision" to "technology licensing" and collaboration; a shift that addresses domestic constraints while simultaneously "reinforcing national sovereignty", and by seeking domestic production agreements for foreign vaccines, Bangladesh transitions from a passive consumer of aid to a sophisticated diplomatic actor, mirroring the ASEAN "agency" model that allows states to reduce long-term dependency on great powers and solidify their "negotiated autonomy" within the broader regional order.

While the prior research has examined vaccine diplomacy as a means of competitive soft power and geopolitical rivalry, much less has been done to understand the extent to which smaller states employ agency within such rivalries. Whereas most research about Bangladesh has considered the state as a balancer amidst rival powers or a receiver of health aid, this research adds to the extant literature by framing Bangladesh's response to vaccine geopolitics as one of "negotiated autonomy," through which Bangladesh used diversification, multilateralism, and selective bilateral engagement to negotiate India-China rivalry and maintain strategic maneuverability during the COVID-19 pandemic. Furthermore, by comparing the vaccine approaches of India and China in Bangladesh, the paper discusses how discrepancies in reliability, continuity, and diplomatic conduct influenced Bangladesh's evolving vaccine procurement and foreign policy behavior.

Vaccine Diplomacy: Politics of Influence

The Politics of Influence in international relations is the process by which states attempt to affect the conduct, inclinations, and choices of other states to satisfy their own national interests. In the early twentieth century, the politics of influence in international relations was mainly accessed through hard power. Military alliances, territorial control, and economic pressure were major instruments particularly during the period of the World Wars and the Cold War. Great powers tried to influence the behavior of other states through coercive means which included both strategic military blocs and ideological competitions. Influence was directly related to the ability of the powerful states to force other states into aligning their foreign policies with their strategic interests. During this period,

influence was, therefore, directly associated with command and coercion, rather than attraction and persuasion (Dahl, 2007).

In the latter half of the twentieth century, social scientists have observed that the ability to influence world politics is no longer confined to the exercise of coercion alone. According to Joseph S. Nye Jr., it has become possible for a country to attain its objectives by shaping the interests and preferences of other actors. Attraction, persuasion, and legitimacy were the methods referred to by him as "soft power". In the latter half of the twentieth century, the rising costs of military interventions have made non-coercive instruments of influence more important in international politics (Nye, 2008). The current global landscape that was primarily centered around post-World War II institutions such as the UN, World Bank, and IMF is now being supplemented with regional platforms and new power groups. These platforms help to expand state influence through diplomatic means without necessarily using military power (Yousofi, 2025).

Therefore, in the 21st century, politics of influence increasingly operates through public diplomacy, economic partnerships, technological leadership, and humanitarian initiatives. Strategies such as development aid, financial assistance, cultural exchanges, media narratives, and initiatives like vaccine diplomacy are examples of how states attempt to build goodwill and expand their influence without direct coercion.

Vaccine diplomacy is the use and delivery of vaccines to improve a country's diplomatic relations with other countries. Countries have exchanged medical devices, professionals and academicians, treatment procedures and healthcare literatures in times of crisis (Bhojraj, 2023). Several scholars view vaccine diplomacy as an important instrument of soft power and nation branding. By providing medical assistance, states are able to project humanitarian commitment and generate international goodwill (M. Bier & Arceneaux, 2020). In this sense, vaccine diplomacy operates within what (Nye, 2008) defines as soft power, the capacity of a country to shape the preferences and behavior of others through attraction rather than coercion or financial incentives.

Scholars highlighted that the distribution of vaccines during the pandemic reflected patterns of geopolitical influence and diplomatic outreach rather than just purely humanitarian considerations. The pandemic presented a unique opening for influential nations such as China and India to extend their sphere of influence in Southeast Asia through this strategy. This approach encompassed the donation and sale of pharmaceuticals as well as cultivating robust strategic partnerships within the region (Chen, 2023).

China's effort of actively supplying its vaccines to countries across Africa, Southeast Asia, and Latin America was widely interpreted as part of its broader strategy to expand its global presence. Additionally, China repaired its international reputation following early criticism regarding the outbreak of COVID-19. Russia also used vaccine diplomacy by promoting its Sputnik V vaccine through bilateral agreements with countries in Latin America, Eastern Europe, and parts of Asia. These efforts were accompanied by diplomatic messaging that emphasized Russia's scientific capability and technological leadership (Leigh, 2021).

These examples show that vaccine diplomacy was not only used as humanitarian aid but gradually became an important tool in the politics of influence. In many cases, vaccine distribution became linked to geopolitical competition and diplomatic interests. Countries used vaccine supplies to strengthen partnerships, improve their global image, and expand their influence in international relations. India's had its own unique stance on vaccine diplomacy through its 'Vaccine Maitri' initiative, reflecting India's approach of using stronger pharmaceutical cooperation and vaccine production capacity to support other nations. Being the pharmacy of the world India particularly focused on the developing countries and its neighbors.

The success of Vaccine Diplomacy illustrates how health assistance can function not only as humanitarian support but also as a strategic tool to build goodwill, reinforce diplomatic ties, and expand a country's influence in global politics.

India-China Relations and Rapprochement

The world's most populous countries, China and India (both Asian nations), are second largest and upcoming third largest economies respectively. They are contending for primacy and influence with the emerging multipolar global order. However, the outside world views the China-India relationship as mere border conflict and unresolved territorial dispute (Bajpae and Jie, 2025). Initially, both shared a cordial relationship, anti-colonial sentiments and had a brotherly bond with each other. Although India and China share long-standing economic and geopolitical interactions, unresolved border disputes and regional competition continue to shape their strategic rivalry

Nevertheless, it is inaccurate to perceive the bilateral relationship through the narrow prism alone. It is rather an indication of broader geopolitical rivalry between two self-perceived 'civilization-states'. Simultaneously, several areas of convergence exist between both countries, which are often missed. Both countries maintain similar worldviews on issues of sovereignty, viewing themselves as leaders

of the Global South and sharing two different civilizational histories. Further both are prominent geopolitical actors with multiple platforms to exhibit power and negotiate with regional partners.

However, exercising the soft power of influencing the regional nations without directly going into war is now a new way of winning. Most prominent instruments are economic help and medical assistance. China has emerged as a leading trade partner and source of foreign investment for the countries in South Asia. It has been imposing economic distress on the region through closed door lending practices which pushes countries into debt trap. Additionally, three countries in the region viz Bangladesh, Pakistan and Sri Lanka received IMF bailouts, while Maldives was at risk of defaulting on its sovereign debt (Bajpae & Jie, 2025). China accounts for more than 70 per cent of Pakistan's external bilateral debt, more than two-thirds of that of Maldives, more than half of Sri Lanka's external bilateral debt and a quarter of that of Bangladesh and Nepal (Bajpae & Jie, 2025, p. 26). India is pitching itself as the reliable alternative for neighbours who have fear of depending on China. (Bajpae & Jie, 2025, p. 27).

China and India, in particular, have been proactive in using medical supplies and vaccines to advance their diplomatic goals. The outbreak of the Chinese-origin viral pandemic in 2019 and the following economic downturn, paved the way for India to reach out with assistance in the healthcare sector to over 150 countries (Chakravarty, 2021). In the UN General Assembly, India declared that it would use its "vaccine production and delivery capacity to help all humanity in fighting the coronavirus crisis". India actively engaged in both bilateral and multilateral vaccine diplomacy. India's vaccine diplomacy focused on donations to its neighbouring countries in South Asia and partners in Southeast Asia and Africa. India airlifted vaccines to Bangladesh, Bhutan, Maldives, Nepal, Myanmar and Seychelles, successively shipments to Afghanistan, Mauritius and Sri Lanka. India supplied Hydroxychloroquine, Remdesivir, Paracetamol, masks, gloves, PPE suits, diagnostic kits and other medical supplies to the neighbourhood (Chakravarty, 2021).

China tried to recover lost ground via vaccine diplomacy in order to overcome the global perception of being responsible for the spread of the pandemic. Its 'Mask and PPE' diplomacy left many countries unhappy about China's mercantilist economic policy by compromising with quality. China categorized its vaccines as 'public goods' and later provided vaccine aid to developing countries in smaller amounts than its commercial contracts (Yang, 2021, p. 2). Brazil, Indonesia and Thailand were among major countries that have received the SINOVAR vaccine. Philippines, Myanmar and Cambodia, an ally of China, which had received a donation of the Chinese vaccine, later reached out to India for more vaccines. This instance reminded India's neighbours that the primary responder in a disaster

situation will always be India. This attempt of vaccine diplomacy created more space for India amidst China's push to expand its influence in South Asia.

Bangladesh Negotiating through Vaccine Diplomacy

“The pandemic is a stark reminder that our fates are interconnected and that no one is secure until everyone is secured.”

The then Hon'ble Prime Minister of Bangladesh, Sheikh Hasina (Sazid, 2022, p. 92)

The COVID era has seen vaccine diplomacy as one of the tools countries used to demonstrate their soft power to other nations. The big countries like India, China, Russia, and the USA have used this instrument to strengthen their bilateral relations with other countries. Thus, vaccine diplomacy became an essential part of global health diplomacy. There have been several studies in different parts of the world that have examined the development and implications of it post COVID 19. According to Sarker & Ghosh (2023), vaccine diplomacy has been defined as a series of actions taken by governments in response to health crises to acquire vaccines, finance, and international cooperation.

The May 2022 report recognised Bangladesh as a pioneer in dealing with the COVID outbreak and placed it at the top among the nations with identical socio-economic structure of South Asia (The Financial Express, 2022). In the case of Bangladesh, this tool has played a significant role in acquiring vaccines for a huge population. The study has shown that Bangladesh has taken a proactive diplomatic stance in order to reduce the effects of the pandemic and maintain public health security. There are several key elements that have influenced the country's diplomacy during the pandemic period. Bangladesh's diplomacy was based on five major components (Sarker & Ghosh, 2023, p. 11):

- Identifying critical intervention areas
- Assessing internal vaccine demand
- Utilising multiple sources of vaccines
- Generating diverse sources of external finance
- Utilising soft power diplomacy.

These elements enabled the country to manage the challenges posed by vaccine shortages and global competition for vaccine supplies.

Initially Bangladesh used diplomatic channels to stay close with its powerful Asian neighbours, India and China. Bangladesh was favourable regarding the potential trial of the Chinese vaccine developed by Sinovac, to be tested on 4,200 Bangladeshi volunteers (*China's COVID-19 Vaccine Trial Approved in Bangladesh*, 2020). Simultaneously, Bangladesh was connecting with India for the procurement of

the Oxford–AstraZeneca vaccine. Bangladesh even offered India to have a vaccine trial inside its territory. However, according to some diplomats this act paved the way for Bangladesh to get secured vaccination (Sarker & Ghosh, 2023, p. 6).

India promised Bangladesh to provide the AstraZeneca vaccines once large-scale production started. With the beginning of the Vaccine Maitri campaign, Bangladesh was one of the first countries to get vaccines (Chawla, 2021). Under the bilateral agreement Bangladesh purchased 15 million vaccines from the Serum Institute in March 2022 (Byron and Habib, 2022).

As a supplementary action regarding the agreement with India, Bangladesh's Foreign Minister called his Indian counterpart as the last resort, requesting to send vaccine doses as a gift (Bhuiyan et al., 2021). The minister said that as part of India's "Neighbourhood First" policy, Bangladesh accorded high priority to the relationship with India. Subsequently, this gift of 20 lakh doses of Covishield vaccine was the biggest such dispatch by India to any of the neighbours. Furthermore, Bangladesh obtained 300 million doses of vaccines from multiple sources, like UNICEF, which distributed approximately 190 million doses to Bangladesh through the COVAX, plus it included purchased, received, and donated vaccines too (*12.94 Lakh People Receive Booster Dose on 1st Day of Campaign*, 2022).

Bangladesh provides a significant case study for understanding how smaller states navigate global vaccine diplomacy. Unlike major powers that produce vaccines domestically, Bangladesh relied largely on international cooperation and diplomatic negotiations to secure vaccine supplies. The other remarkable aspect of vaccine diplomacy in Bangladesh is its engagement in multilateral approaches, including the COVAX facility.

Table 1

Source country	Vaccine type	Year	Approximate dose received	Mode of diplomacy
India	Oxford - AstraZeneca (Covishield)	2021	~12 million doses	Bilateral vaccine diplomacy under the “Vaccine Maitri initiative
China	Sinopharm	2021-2022	~70 million doses	Bilateral diplomatic agreement and donation
COVAX Initiative	Pfizer, Moderna, Astra	2021-2022	~100 million doses	Multilateral global vaccine cooperation

Source: (COVID-19 Cases, World, WHO.)

The COVAX facility is an initiative of various international organisations, including the World Health Organisation (WHO) and Gavi, that aimed to ensure the equitable distribution of COVID-19 vaccines worldwide, particularly in developing countries (COVID-19 Cases, World, n.d.). In this regard, Bangladesh received millions and many more of vaccines through the COVAX facility, which was crucial in supporting its vaccination drive. Scholars highlight that Bangladesh pursued a pragmatic and diversified approach in its diplomacy by engaging various international actors simultaneously. These approaches included bilateral relations with India and China, negotiations with Russia over vaccine production cooperation, and engaging in multilateral approaches through the COVAX facility under the support of the WHO and other international organisations. Consequently, Dhaka was successful in vaccinating 569,825 individuals with the fourth dose as well. More than 90% of the population has gotten at least one dose of the COVID-19 vaccine in Bangladesh.

However, one major challenge was the disparity in the availability of vaccines between developed and developing countries. According to scholars, developed countries managed to acquire a substantial number of vaccines by purchasing them in advance, leaving behind a few for developing countries (Hossen et al., 2025). This act has been termed as vaccine nationalism, which imposed major hindrance for countries that relied on imported vaccines. In the case of Bangladesh, the country initially relied on imported vaccines from India; however, India faced a halt in the export of vaccines due to a surge in demand domestically.

Despite these challenges, Bangladesh demonstrated considerable diplomatic flexibility during the COVID-19 pandemic. By maintaining balanced relations with multiple global powers and participating in international cooperation mechanisms, the country was able to secure vaccines from various sources. Scholars suggest that this multi-directional diplomatic approach helped Bangladesh reduce dependence on a single supplier and ensured a more stable vaccine supply (Hossen et al., 2025). This strategy reflects Bangladesh's broader foreign policy principle of maintaining friendly relations with different major powers while prioritising national interests.

In addition to diplomatic negotiations, Bangladesh also implemented domestic strategies to manage vaccine distribution and public health measures. For example, the government of Bangladesh introduced digital platforms such as the *Surokkha* application to facilitate vaccine registration and prioritise vaccination for frontline workers and vulnerable populations (Sarker & Ghosh, 2023, p. 12). These initiatives contributed to the country's relatively successful vaccination campaign during the pandemic.

Table 2: India-China Comparative framework with consistent indicators.

Variable	India	China
Vaccine Volume Supplied	Substantial early supply: 2 million doses as grant and 5 million doses delivered under procurement (Jan 2021); agreement for 30 million doses via Serum Institute (Molla, 2021; Sakib, 2021; <i>India Gifts...</i> , 2021)	Limited initial aid relative to commercial commitments; larger cumulative global distribution through contracts rather than grants (Yang, 2021)
Pricing Strategy	Low-cost, quasi-subsidized pricing (~US \$4 per dose) under bilateral agreement, enhancing affordability (Molla, 2021)	Predominantly commercial pricing; vaccines framed as "global public goods" but distributed largely through paid agreements (Yang, 2021)
Delivery Timelines	Rapid first-mover advantage with early approval and dispatch (Jan 2021); supply disruption following domestic second wave (Sakib, 2021; Hossen et al., 2025)	Delayed initial entry but greater continuity and responsiveness after supply gaps emerged (Yang, 2021)

Conditionalities / Terms	Combination of grants and commercial deals under “Neighbourhood First” policy; minimal explicit conditionalities (Bhuiyan et al., 2021)	Commercially driven agreements with implicit strategic expectations , linked to broader geopolitical positioning (Yang, 2021)
Scale of Distribution	Primarily regional focus (South Asia, parts of Africa and Southeast Asia); assistance extended to 150+ countries in early phase (Chakravarty, 2021)	Global scale outreach across Asia, Africa, and Latin America; integrated with wider economic and diplomatic networks (Yang, 2021)
Mode of Supply	Hybrid model: grants + commercial exports + medical aid (vaccines, drugs, PPE, diagnostics) (Chakravarty, 2021)	Predominantly commercial supply , supplemented by selective donations (Yang, 2021)
Strategic Intent	Soft power projection and regional leadership; positioning as a trusted and reliable alternative to China (Bajpae & Jie, 2025)	Restoration of global image and expansion of geopolitical influence through health diplomacy and economic leverage (Yang, 2021; Bajpae & Jie, 2025)
Reliability Perception	Initially high credibility due to speed and proximity; eroded by export suspension , exposing supply vulnerability	Initially questioned (quality concerns during PPE phase), but improved due to continuity and availability
Regional Reception (Bangladesh)	Strong positive reception in early phase; dependence reinforced through agreements and diplomatic proximity; later tempered by supply disruption	Pragmatic and strategic acceptance ; engagement included vaccine trials and procurement diversification despite cost concerns
Impact on Bangladesh’s Strategy	Created early dependence, revealing risks of single-source reliance	Enabled supply diversification and bargaining leverage , reinforcing Bangladesh’s negotiated autonomy

Table 2 is based on secondary sources analyzing vaccine diplomacy during COVID-19. Vaccine examples include Covishield and Sinopharm COVID-19 vaccine / Sinovac COVID-19 vaccine. It provides a comparative overview of India and China’s vaccine diplomacy strategies in Bangladesh, highlighting key differences in delivery models, strategic intent, and their implications for Bangladesh’s negotiated autonomy

From the above discussion it is evident that Bangladesh's vaccine diplomacy was a mixed type of diplomatic negotiation, international cooperation, and domestic policy responses. In this case, even though Bangladesh was likely to face some huge problems due to its limited health care infrastructure and dependence on external sources of vaccines, its diversified diplomatic strategy helped it access vaccines and advance its position in international health governance.

Research Methodology

This study adopts a qualitative case-study approach to analyse how Bangladesh navigated India-China geopolitical competition through vaccine diplomacy during the COVID-19 pandemic. Process tracing is employed as the principal method to analyze specific diplomatic moves, agreements, and supply chains that influenced Bangladesh's foreign policy behavior from 2020 to 2022.

The unit of analysis comprises vaccine-focused diplomatic engagements with Bangladesh, India, China, and international organizations like COVAX, including bilateral vaccine agreements, diplomatic declarations, procurement discussions, donation pledges, export disruptions, and technology transfer talks.

The study relies on both primary and secondary sources. Primary materials include official government statements, speeches by political leaders, Ministry of External Affairs releases, WHO publications, and bilateral agreements. Secondary sources include peer-reviewed journal articles, policy reports, ORF analyses, vaccination datasets, and selected newspaper coverage relevant to South Asian vaccine diplomacy.

Data collection sources are chosen according to the following standards: institutional reliability, relevancy to Bangladesh's vaccine diplomacy, and the empirical verifiability of claims. Unverifiable opinion blogs were avoided unless directly related to diplomatic communications.

Data analysis for this research was conducted using thematic analysis on four comparative indicators that included vaccine dosage distributed, pricing model, delivery schedule, and conditionalities adopted. The study compares India's Vaccine Maitri project with China's strategy of vaccine diplomacy, and it explores the efforts made by Bangladesh to diversify its partnerships amid shifting regional dynamics.

The research methodology gives this study the opportunity to conduct vaccine diplomacy analysis not only from the humanitarian aspect but also from the strategic and geopolitics perspective.

Key Findings

- The study points out the importance of international organizations and global efforts to assist the vaccination drive of Bangladesh. The role of multilateral cooperation was important to avoid bilateral diplomacy and unilateral supremacy of India or China in South Asia.
- China's approach was global and commercially driven. It invested heavily in domestic R&D, exporting technology and manufacturing to secure massive international orders (Yang, 2021, p. 04). It can be perceived that China's outreach is a tool for geopolitical pressure rather than a medical necessity.
- India's strategy primarily relied on aid-based distribution rather than large-scale commercial sales. Its role was more of a low-cost manufacturer for foreign-developed vaccines hence the diplomatic reach of India was more localized and operationally constrained compared to China's.
- The research study has brought out the fact that the success of Bangladesh's vaccine diplomacy was not entirely reliant on its foreign relations but was also aided by domestic governance practices. Digital registration, targeting vulnerable populations, and conducting vaccination drives were crucial during COVID era.
- Unlike traditional military or resource competition, this diplomacy race provided benefits to recipient nations by allowing them to diversify their supplies and negotiate between the two powers.
- The paper indicates that developing states can accept investments from China without negatively impacting the health support they receive from India. This would imply that states with strong economic relations with India but who are increasingly influenced by China would still benefit from India's vaccine diplomacy (Brink, 2023, p. 22).
- Bangladesh's vaccine procurement trajectory reflected a transition from dependence-based alignment toward diversified hedging involving India, China, and multilateral mechanisms such as COVAX.
- The comparative analysis indicates that while India generated rapid regional goodwill through timely assistance, China maintained greater continuity through sustained vaccine outreach and commercial engagement.

- Bangladesh leveraged competing vaccine channels to expand its strategic options within asymmetric regional power competition.

Conclusion

In an increasingly competitive geopolitical environment, the strategic influence generated through vaccine diplomacy remains largely transitory in nature. Amidst the power politics of major states, India and China extended their sphere of influence and provided medical assistance to developing nations thereby portraying humanitarian aid. Bangladesh, being India's neighbour had its own unique challenges of navigating from humanitarian necessity to strategic survival mechanism. The paper examines how Bangladesh negotiated between India and China through vaccine diplomacy during the COVID-19 pandemic. Bangladesh adopted both bilateral and multilateral diplomatic strategies by diversifying vaccine procurement beyond a single source of dependence. It had the opportunity to take advantage of the rivalry and expand its opportunities instead of relying on a single country. The study contributes to the broader literature on small-state agency and geopolitical competition by demonstrating how developing states can strategically navigate asymmetric power rivalries during global crises. It also opens the scope for further study of Personal Protective Equipment (PPE) kits, Remdesivir injections, Oxygen cylinders and other medical aid countries provided during those times. The success of these diplomatic efforts is highly sensitive to the internal governance and shifting domestic politics of recipient nations. Following the change in interim leadership of Bangladesh, its domestic government has strained the relations with India. Further, this study has not covered the influence of domestic politics over a nation's foreign diplomacy, which could be studied in future research.

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